

--SUMMARY--

Decision No. 20/19

15-Jan-2019

G.McCaffrey

- Accident (occurrence)

No Summary Available

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- WSIA

Other Case Reference

- [w1019n]

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 20/19

BEFORE: G. McCaffrey: Vice-Chair

HEARING: January 7, 2019 at Toronto
Oral

DATE OF DECISION: January 15, 2019

NEUTRAL CITATION: 2019 ONWSIAT 129

DECISION UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) A. Rivet
dated March 16, 2017

APPEARANCES:

For the worker: M. Farago, Union

For the employer: M. Rocca, Paralegal

Interpreter: N/A

REASONS

(i) Introduction and issues

[1] The worker and employer appeal a decision of the Appeals Resolution Officer (ARO) A. Rivet dated March 16, 2017.

[2] The employer appeals:

1. The initial entitlement granted for a right knee injury claimed to have occurred on March 3, 2015.
2. The entitlement granted for a September 23, 2015 recurrence of the right knee injury in the March 3, 2015 claim.

[3] The worker appeals:

1. The initial entitlement for a right knee injury occurring on September 23, 2015.

(ii) Background

[4] The following are the basic facts.

[5] The worker began his employment as a drywall installer with the employer in 2006. He claims that on March 3, 2015, at age 59, he slipped on an icy step coming out of an on-site bathroom, fell to the ground, and twisted his right knee. He sought medical attention at hospital emergency later that day. The worker was reportedly off work between two and three weeks, but did not initiate a WSIB claim.

[6] The worker claims that on September 23, 2015 he was installing drywall in a garage, slipped on the rungs coming down from the scaffolding, and twisted his right knee. He did not return to work, and did not immediately initiate a WSIB claim. When the worker initiated a WSIB claim, on October 29, 2015, he reported both incidents.

[7] The worker had right knee surgery on February 29, 2016 which included debridement of osteoarthritic changes in two compartments, and medial and lateral meniscectomies.

[8] The ARO decision of March 16, 2017 granted initial entitlement for the March 3, 2015 right knee injury, and the September 23, 2015 incident as a recurrence of the March 3, 2015 injury. The ARO concluded proof of a new accident on September 23, 2015 had not been established.

(iii) Law and policy

[9] As the worker claims he was injured in 2015, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[10] An “accident” is defined in section 2(1) to include:

- (a) a wilful and intentional act, not being the act of the worker,
- (b) a chance event occasioned by a physical or natural cause, and
- (c) disablement arising out of and in the course of employment;

[11] General entitlement to benefits is governed by section 13:

13(1) A worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan.

(2) If the accident arises out of the worker's employment, it is presumed to have occurred in the course of the employment unless the contrary is shown. If it occurs in the course of the worker's employment, it is presumed to have arisen out of the employment unless the contrary is shown.

[12] The statutory presumption set out in section 13(2) does not apply to an injury by disablement. See, for example, *Decisions No. 268* and *42/89*.

[13] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*. The standard of proof in workers' compensation proceedings is the balance of probabilities.

[14] I have considered the policies the Board has stated apply to the subject matter of this appeal, pursuant to section 126 of the WSIA.

[15] *Operational Policy Manual* (OPM) Document No. 15-02-01, "Definition of Accident", describes a chance event as "an identifiable unintended event which causes an injury", an injury itself is not a chance event. The policy defines a disablement as "a condition that emerges gradually over time" or "an unexpected result of working duties."

[16] OPM Document No. 11-01-01, "Adjudicative Process", states that an allowable claim must have five points: an employer, a worker, personal work-related injury, proof of accident, and compatibility of diagnosis to accident history. OPM Document No. 11-01-01 provides the following guidelines for determining proof of accident:

Proof of accident

Decision-makers may consider the following when examining proof of accident,

- Does an accident or disablement situation exist?
- Are there any witnesses?
- Are there discrepancies in the date of accident and the date the worker stopped working?
- Was there any delay in the onset of symptoms or in seeking health care attention?

(iv) Testimony

[17] The worker testified:

- He worked installing drywall off and on for about 35 years and always worked through the union.
- He played baseball and hockey when he was younger but suffered no injuries, and specifically, no knee injuries.
- Other than surgery for a blood clot in his lower leg at around age 15 he had no surgeries prior to March 3, 2015.

- Prior to 2015 he had dealt with the union disability person when he received benefits after falling from a ladder at home in 2008, fracturing his T11 and T12 vertebrae, and being off work for about a year. Surgery was not required.
- Prior to March 3, 2015 he had been experiencing right knee pain for some time and as a result had been taking three Advil a day for a couple of years.
- MT had been his drywall partner for about one year prior to March 3, 2015. They lived in the same area and drove to work together. MT drove.
- On March 3, 2015 MT drove them to work. They were dry walling apartments several floors up in a commercial/residential project. Later in the morning, just before they normally took a break he went down to the ground to use the portable washroom which was a heated trailer with two metal steps. Upon leaving the washroom he slipped on an icy step, and fell to the ground twisting his right knee. The elevator guy saw him fall. VB, the health and safety manager for the general contractor was close by and came to help him up. The general contractor job supervisor, T, also came over.
- He knew there was something wrong with his knee, and his company foreman, RD, offered to drive him to the local hospital. He went up to tell MT that he had injured his knee and that RD was taking him to the hospital. They went to the hospital in RD's van, a distance of approximately 3 km. RD left him at the entrance to the emergency department. Upon entering the emergency department he took a number. He was unsure how long he waited before being processed.
- He had an x-ray of his right knee, was given an appointment for an MRI, and had his right leg placed in a plastic cast with Velcro closures. He called MT to advise he would get home on his own, left the hospital around 3:30 p.m. to 4:00 p.m., and then took several buses to get home.
- With respect to the Emergency Triage Record indication that there was a red and swollen area on his right lower leg that was not healing, the worker indicated that he had a rash and asked them to look at it. When directed to the indication "struck right lower leg several weeks ago" he acknowledged that he may have bumped it.
- The following day, March 4, 2015 he returned to the jobsite to give VB the WSIB forms from the hospital. He also saw RD and advised he would be off work. RD asked "how do you want to do it." As he thought he would only be off a couple of weeks he did not complete any WSIB forms for RD.
- He had no prior WSIB claims however had heard stories, did not want to do "modified work" sitting in the shop doing nothing, and was worried having a WSIB claim would affect his future employment.
- He had no recollection of returning to the hospital emergency department on March 4, 2015 to have a WSIB form filled out, even after being directed to the March 4, 2015 Emergency Triage Record and the Health Professionals Report (Form 8) dated March 4, 2015.

- He returned to the same hospital on March 20, 2015 for an MRI. Although the MRI findings suggest previous meniscectomies and a “chronic complete ACL tear” the worker was adamant that he had no previous knee injuries or surgeries.
- He was off work for two to three weeks. MT continued working. Due to the nature of the invoicing process for piecework he continued to receive cheques from the employer while he was off.
- While he was off he went to physiotherapy twice but then did not go back as he did not feel it was helping his knee. He wore the brace/cast for about ten days.
- When he returned to work he had to provide a doctor’s note to the general contractor in order to be allowed on the site. The note may have been from Dr. White.
- He was unsure which doctor provided the note as he has always attended the same medical clinic since at least 2008, however, as doctors retire and new doctors join there are transition periods. As it is the same clinic whichever doctor he sees has his complete chart.
- MT continued to drive them to work, and they continued to work as partners. Later they moved on to other jobs, still with the employer but had different foremen.
- His right knee remained sore, he was not as mobile, and as a result his output and earnings were affected. A number of times between March 2015 and September 2015, he saw a doctor at the clinic, probably Dr. Saksen.
- In September 2015 the worker and MT were dry walling garages in three houses for the employer. MT had to leave the job on September 22, 2015 due to an illness in his family.
- As they were working from scaffolding and the job required two people he got a friend, R, to assist him on Wednesday September 23, 2015. While climbing down from the scaffolding his right foot slipped off the last rung causing him to again twist his right knee. As the foreman, G, was not at the jobsite he called him to let him know the work would not be finished that day. G was very upset.
- He went to his doctor the next day, September 24, 2015, or possibly on Friday September 25, 2015. He thinks he saw Dr. White, and would have told him that he twisted his knee at work.
- He could not explain why the earliest medical report from his medical clinic was from an October 1, 2015 visit, in which Dr. Saksen makes no reference to a visit the previous week, or the September 23, 2015 incident.
- After September 23, 2015 he attempted to return to work many times, several times with MT, but could not do it. As result he called the union benefits person. She suggested it sounded like a WSIB case. Although he had not want to claim WSIB benefits, as he remained unable to work he filed a WSIB claim.
- Dr. Mackenzie performed the right knee arthroscopic surgery on February 29, 2016.
- He had a right knee replacement in June 2016.
- He had a left knee replacement in April 2017.

- There is no other reason he has been off work since September 23, 2015 other than the right knee pain and the subsequent surgeries to both knees.
- He has received union disability benefits and social assistance benefits but not ODSP or CPP benefits.

[18] The coworker, MT, testified:

- He and the worker are friends and were drywall partners the majority of the time since about 2011.
- He and the worker would drive to work together, the worker was a passenger. The worker did not complain about any physical problems prior to March 3, 2015 other than mentioning an old back injury. Because they do a physical job they would take the occasional Advil however he never saw the worker taking pain medication on a regular basis prior to March 3, 2015. He doubted the worker would take pain medication regularly as he was aware the worker had an unrelated medical condition.
- On March 3, 2015, as it was approaching their usual break time between 9:30 a.m. and 10:00 a.m. the worker indicated he was going down to the washroom.
- Eventually the worker came back, advised that he had slipped and twisted his knee coming out of the washroom, and that RD was taking him to the hospital.
- He subsequently looked out a window and saw the worker leaving in RD's van. Later on March 3, 2015 the general contractor health and safety person, asked him to make sure the worker saw him the next day regarding paperwork.
- That evening he spoke to the worker on the phone regarding the paperwork. They drove to work together on March 4, 2015 and as the worker was wearing a brace on his leg, he dropped him as close to the office as he could.
- Around 9:00 a.m. he drove the worker to the hospital the worker had been at the previous day so that the worker could obtain some more paperwork. The worker took the bus home again that day.
- He continued working. When he was talking to T after March 3, 2015, T asked how the worker was doing.
- The worker returned to work after two and a half or three weeks. Upon his return the worker was favouring his right knee a bit, and was cautious regarding the scaffolding, so MT did most of the climbing.
- After he returned to work the worker was taking one to two Advils per day or one prescription pill.
- He was not at work on September 23, 2015 so did not witness that injury.
- The worker did attempt to return to work with him on one occasion in late October but only lasted one to two hours.

(v) Analysis

[19] For the following reasons I find that evidence before me supports that the worker sprained his right knee at work on March 3, 2015. I further find that neither proof of an accident or a recurrence on September 23, 2015 has been established.

(a) The time limit to claim

[20] The employer's representative submits that the worker did not meet the section 22 requirement to submit a claim within six months of March 3, 2015.

[21] The employer's representative referred to OPM Document No. 15-01-03, "Workers' Requirement to Claim and Consent" which indicates in part:

Policy

The WSIB only issues one benefit payment (up to two weeks of loss of earnings benefits) to workers who are entitled to benefits under the insurance plan, but who have not met the claim and consent requirements. No further benefits are provided unless the worker meets the requirements.

As soon as possible after an accident, workers must file a claim for benefits. They must also consent to disclose their functional abilities information, which is provided by the treating health professional.

A claim must be filed within six months of an accident or, in the case of an occupational disease, within six months of the worker learning of the disease. The WSIB may extend the six-month deadline, or waive the dual requirement altogether if in the WSIB's opinion it is just to do so.

If the worker does not file the claim for benefits, or consent to the disclosure of functional abilities information within the six-month deadline, the WSIB does not provide benefits unless, in its opinion, it is just to do so.

Purpose

The purpose of this policy is to outline how workers can meet the requirements for claiming benefits, and to identify when the WSIB may extend the time to claim and consent.

[...]

Accident not reported by employer

Independent of the worker's requirements to claim and consent, employers must report accidents to the WSIB, see 15-01-02, Employers' Initial Accident-Reporting Obligations. Therefore, the WSIB will accept a worker's claim past the six-month deadline if

- the worker does not file a claim by the six-month deadline, but
- the employer did not report the accident, and
- Regulatory Services determines that the employer created a coercive workplace environment which resulted in the worker waiving, or forgoing, any benefits to which the worker was entitled.

Once Regulatory Services makes this determination, the worker has thirty calendar days to file the claim. In these cases, no benefits are provided until the worker meets the dual requirements.

[22] The worker's representative submits that the Board found that the employer was aware of the worker's injury, and as a result the employer was obligated to report the accident to the

WSIB. He submits the worker and employer obligations are balanced and noted the ARO accepted the worker had met his reporting obligation, and that the accident had occurred.

[23] OPM Document No. 15-01-02, "Employers' Initial Accident-Reporting Obligations" states in part:

Policy

When notice is required

Employers **must** report a work-related accident to the WSIB if they learn that a worker requires health care and/or

- is absent from regular work
- earns less than regular pay for regular work (e.g., part-time hours)
- requires modified work at less than regular pay
- requires modified work at regular pay for more than seven **calendar** days following the date of accident.

[...]

When notice is not required

Employers are not required to report a work-related accident if the worker

- receives only first aid
- receives first aid and requires modified work at regular pay for seven calendar days or less, following the date of accident
- does not receive first aid, but requires modified work at regular pay for seven calendar days or less, following the date of accident.

[24] I find that the delay in establishing a WSIB claim until early November 2015, ten months after March 3, 2015, should not be a bar to entitlement being considered given the particular circumstances of this appeal.

[25] Approximately five weeks after September 23, 2015, when he stopped working, the worker initiated a WSIB claim on the advice of the union disability benefit person. The worker's completed Worker's Report of Injury/Disease (Form 6) is dated October 29, 2015 and refers to both the March 3, 2015 and September 24, 2015 incidents. The WSIB established two claims.

[26] The worker testified that he had not wanted to establish a WSIB claim partially over concern it would affect his opportunities for future employment.

[27] The company foreman, RD, did not attend the January 7, 2019 Tribunal hearing. However, he provided testimony at the March 14, 2017 ARO hearing. In her decision, the ARO found RD's testimony was contradictory, but found that it confirmed RD was aware the worker sustained a March 3, 2015 injury and that he was told by the safety representative that there was an incident. I agree with the ARO's finding that the worker had met his reporting obligation when he advised the employer of his injury, but that the employer took no action in reporting the injury to the WSIB.

[28] The employer's representative noted that in an April 13, 2016 letter to the union her firm advised that on April 12, 2016 the safety manager, VB, confirmed there was no report or mention of an alleged accident involving the worker on March 3, 2015. I assign no weight to

that statement as it is untested and is inconsistent with the testimony of RD as recorded by the ARO, and the testimony of the worker.

[29] Therefore, I find that the worker met his obligation to report the accident to his employer. I also find that given the circumstances of this appeal, the WSIB appropriately extended the time for the worker to establish the March 3, 2015 claim.

(b) March 3, 2015

[30] I find that the worker sprained his right knee as a result of a work accident on March 3, 2015 for the following reasons.

[31] As is often the case with disputed initial entitlement cases there are no statements from direct witnesses to the actual incident. Accordingly, one must have regard for the circumstantially compelling evidence. In this appeal it has not been disputed that the worker was on site and installing drywall during the morning of March 3, 2015. The worker has consistently reported that he slipped and fell on icy steps causing him to twist his right knee. While the extent of the immediate reporting of the incident to the employer and general contractor is in dispute, I find that at the very least, the worker reported to his partner, MT, that there had been an injury.

[32] I find that the worker was unable to continue working. While it is disputed whether RD drove the worker to the hospital there is no dispute emergency medical attention was sought. The Emergency Triage Record indicates that at 12:40 p.m. he reported a slip/fall on ice onto metal steps at work.

[33] MT testified the worker was wearing a leg cast/brace the following day when he drove the worker to the job site to provide the safety manager with paper work. MT also testified he drove the worker back to hospital emergency later that morning. The Emergency Triage Record of March 4, 2015 confirms that at 9:20 a.m. the worker returned to have a WSIB form filled out. A Health Professional's Report (Form 8) dated March 4, 2015 which indicates "slip and twisted knee" and "off work until Mar 13" is contained in the record. The company foreman, RD, confirmed that by March 4, 2015 he and the general contractor safety manager were aware an incident had occurred on March 3, 2015.

[34] The employer's representative questioned the time gap between the Emergency Triage Record indication that the worker reported that the injury occurred at 8:30 a.m., and the Triage time of 12:40 p.m. I note MT estimated that the worker did not leave to go to the washroom until just before their normal morning break, around 9:30 a.m. to 10:00 a.m. MT also testified he then continued to work for a considerable time before the worker returned to advise that he had hurt his knee and was going to the hospital. The worker testified that upon arrival at the Emergency Department he took a number and waited before being seen by Triage. As a result, I find that there is not an unexplained gap between the time of the injury and the worker's arrival at the hospital.

[35] Therefore, when considering the circumstantial evidence, I find that on the balance of probabilities the worker sprained his right knee on March 3, 2015 as result of a slip and fall at work.

[36] As noted by the ARO there are substantial gaps in the medical documentation contained in the record. For example, the March 3, 2015 emergency record indicates the worker was

referred for an orthopaedic assessment to occur on March 12, 2015 at 10:20 a.m. That orthopaedic report is not contained in the material before me.

[37] Also, the March 20, 2015 MRI findings were consistent with the worker having previous knee surgeries and a chronic complete tear of the anterior cruciate ligament. Although the worker was adamant that he had no prior knee surgery, I give significant weight to the findings as the report indicates they were made despite the fact that “none is described by the patient or the requesting physician.” While the February 29, 2016 surgical report does not address the prior surgical findings it indicates “the ACL, PCL were normal”. In the absence of the relevant medical documentation, including any other imaging, the disparity between the March 20, 2015 MRI findings and the February 29, 2016 surgical findings cannot be resolved. However, I find that the worker does not have entitlement for a chronic complete ACL tear or prior menisci damage identified on the March 20, 2015 MRI.

[38] Although the worker testified he was required to provide clearance from his physician upon his return to work two and a half to three weeks after March 3, 2015, there is again no medical documentation before me regarding the worker’s absence from work in March 2015. As a result the nature and duration of any LOE benefit entitlement during those two and a half to three weeks is returned to the WSIB for adjudication.

(c) September 23, 2015

[39] The ARO granted entitlement for the alleged September 23, 2015 incident as a recurrence of the March 3, 2015 right knee injury. The worker’s representative submits that he is satisfied with the ARO’s acceptance of the recurrence. He submits the worker is only claiming the September 23, 2015 incident as a new injury if entitlement for the recurrence is withdrawn.

[40] I find that neither proof of an accident or a recurrence on September 23, 2015 has been established.

[41] OPM Document No. 15-02-05, “Recurrences”, indicates in part:

Policy

A worker may be entitled to benefits for a recurrence of a work-related injury/disease if the worker experiences a significant deterioration that

- does not result from a significant new incident/exposure, and
- is clinically compatible with the original injury/disease.

[...]

New incident/exposure

If the significant deterioration results from a significant new incident/exposure (work-related or not) a recurrence is not considered. Instead, if the significant new incident/exposure is work-related, a new claim is considered.

If the significant deterioration occurs when there is no new incident/exposure or results from an insignificant new incident/exposure (work-related or not) a recurrence is considered.

A significant new incident/exposure is one of some consequence or importance (e.g., falling from a ladder). An insignificant new incident/exposure is one of negligible consequence or importance (e.g., reaching for an object on a shelf).

Clinically compatible

To establish that the significant deterioration is clinically compatible with the original injury/disease, the decision-maker must determine that

- the body parts and/or functions affected now are the same as, or related to, those affected by the original injury/disease, and
- there is a causal link between the significant deterioration and the original injury/disease.

To make these determinations, the decision-maker considers the nature and severity of the significant deterioration, the original injury/disease and any relevant non-work-related conditions that are present.

The decision-maker may also consider whether a worker has experienced continuing symptoms since the original injury/disease. Generally, continuing symptoms are an indicator of a causal link, though they are not required to establish a causal link.

Indicators of continuing symptoms may include

- continuing clinical treatment
- continuing workplace accommodations, or
- evidence that continuing symptoms were reported to health care providers, supervisors or co-workers on an ongoing basis.

If the decision-maker determines that the existing evidence does not clearly demonstrate whether the significant deterioration is clinically compatible with the original injury/disease, the decision-maker may seek a clinical opinion to assist in making this determination.

[42] In this appeal there is no medical documentation before me establishing continuing treatment between the March 3, 2015 injury and September 23, 2015. Although the worker testified he continued to see his family physician approximately every four to six weeks, and in March 2017 the ARO noted those reports were not contained in the record, there remains no medical documentation establishing continuing medical treatment in the documentation before me.

[43] The worker agrees that he returned to his regular employment installing drywall between his return to work, likely in late March 2015, and September 2015. I find the testimony of MT that the worker was taking two to three pills per day during this period does not establish that there was an ongoing right knee condition related to the March 3, 2015 fall as the worker testified that he was taking three Advil pills per day for knee pain prior to the March 3, 2015 fall.

[44] With respect to the September 23, 2015 incident, the worker testified he was working with a friend, R, as MT had been called away the previous day due to a family emergency. There is no statement in the record from R. Although the worker testified that on September 22, 2015 he contacted his friend, R, to ask for help completing the drywall work, on November 17, 2015 the worker advised the Board that he did not have R's contact information (Board Memorandum # 5).

[45] The worker also testified that he telephoned his foreman, G, to advise the drywall work in the garage would not be completed. There is no statement from G in the material before me. As a result there is no confirmation whether the worker reported a slip from the bottom rung of the scaffolding to the employer on September 23, 2015.

[46] Although the worker has consistently testified that he sought medical attention at his family physician's clinic on September 24, 2015, or possibly Friday, September 25, 2015, the earliest clinical note provided by the medical clinic is dated October 1, 2015. In that typed clinical note Dr. Sacksen does not refer to a visit the previous week, make any reference to the worker reporting a September 23, 2015 incident, or indicate there were acute symptoms such as swelling of the right knee.

[47] The worker was subsequently assessed by Dr. McKenzie, orthopaedic surgeon, on November 23, 2015. Dr. McKenzie indicates the worker "has had knee pain for quite some time." There is no reference to any knee injury.

[48] The pre-surgery imaging, dated February 26, 2016 indicates:

There is extensive tricompartmental osteoarthritis present. Severe joint space narrowing is present medially. No acute abnormality present.

[49] Dr. McKenzie performed the arthroscopic right knee surgery on, February 29, 2016.

[50] Therefore, with respect to the alleged September 23, 2015 incident, I find there is no proof in the documentation before me that a right knee injury occurred on that date.

[51] Furthermore, I find that the documentation before me does not contain that the elements that would establish that continuing symptoms related to the original March 3, 2015 right knee sprain were present. There is no confirmation of continuing clinical treatment, neither the worker nor employer indicated there were continuing workplace accommodations, neither the worker nor employer indicated continuing symptoms were reported to the employer, and there is no confirmation that continuing symptoms related to the March 3, 2015 fall were reported to health care providers.

[52] Based on the worker's testimony regarding knee pain and the amount of medication he was taking prior to March 3, 2015, I find MT's testimony that the worker had difficulties due to right knee pain, and was taking medication prior to September 23, 2015 is not in itself persuasive that those symptoms arose directly from the March 3, 2015 sprain and not the symptomatic underlying condition.

[53] With respect to the clinical compatibility of the worker's knee symptoms subsequent to September 23, 2015 and the March 3, 2015 sprain, I find there is no medical documentation before me upon which to make that finding. The worker testified that he had been experiencing knee pain for a couple of years prior to March 3, 2015. The February 26, 2016 imaging, included above, confirmed extensive osteoarthritis. The worker testified that he a total right knee replacement in June 2016, followed by a total left knee replacement in April 2017. Based on the limited medical documentation before me I find the worker's right knee symptoms in September 2015 were consistent with an ongoing non-compensable condition as opposed to the March 3, 2015 sprain. I was not taken to any medical evidence of significance linking the worker's knee pain to the March 3, 2015 sprain in a causative way.

[54] Therefore, on the balance of probabilities I find that proof of an accident occurring on September 23, 2015, or in the alternative, a recurrence of the March 3, 2015 right knee sprain occurring on September 23, 2015 has not been established. Accordingly the worker does not have entitlement in the March 3, 2015 claim for the alleged September 23, 2015 recurrence, and he does not have initial entitlement for a new right knee injury on September 23, 2015.

DISPOSITION

[55] The employer's appeal is allowed in part as follows:

1. The worker has entitlement for a right knee sprain arising from his fall at work on March 3, 2015.
2. The worker does not have entitlement for a chronic complete ACL tear or prior menisci damage identified on the March 20, 2015 MRI.
3. Entitlement for the claimed September 23, 2015 incident as a recurrence of the March 3, 2015 work accident is rescinded.

[56] The worker's appeal is denied. The worker does not have initial entitlement for the claimed right knee injury on September 23, 2015 as a new accident.

[57] The nature and duration of benefits in the March 3, 2015 claim flowing from this decision are returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: January 15, 2019

SIGNED: G. McCaffrey